

BS™

(Co-Trimoxazole) Tablets

Composition

BS is Co-trimoxazole, a 1:5 combination of Trimethoprim and Sulphamethoxazole.

Each tablet contains Trimethoprim 80 mg and Sulphamethoxazole 400mg

Properties

BS has a broad spectrum antibacterial action that covers a wide range of gram-positive and gram-negative organisms such as streptococci, staphylococci, pneumococci, *Haemophilus influenzae*, *Neisseria*, *E.coli*, *Proteus mirabilis*, *P. vulgaris*, *Bordetella*, *Salmonella*, *Klebsiella*, *Aerobacter*, *Shigella*, *Vibrio cholerae*, *Brucella*, *Ps. cepacia*, *Pneumocystis carinii*, *Yersinia* and *Nocardia*. It may be effective against *Burkholderia pseudomallei*.

Indications

Treatment of infections caused by susceptible organisms, as in: respiratory tract infections, urinary tract infections, gastro-intestinal tract infections, including typhoid and paratyphoid fever; cholera; skin and soft tissues infections; brucellosis (generally in combination with other agents), septicaemia due to sensitive organisms, nocardiosis, mycetoma (except when caused by the true fungi). For South American blastomycosis, **BS** is suppressive but not curative.

Dosage and Administration

To be administered orally.

Adults and children over 12 years of age: 1 to 3 tablets taken twice daily in the morning and evening.

For uncomplicated gonorrhoea: 4 tablets taken once in the morning and a further 4 tablets in the evening for two days.

In acute infections, **BS** should be given for at least five days or until the patient is free from symptoms for two days.

Contraindications

BS should not be used in patients with marked liver parenchymal damage; severe renal insufficiency; or patients with serious megaloblastic bone marrow; or patients with documented megaloblastic anemia due to folate deficiency.

BS is contraindicated in patients with a known hypersensitivity to trimethoprim or sulphonamides, in patients with a history of drug-induced immune thrombocytopenia with use of trimethoprim and/or sulphonamides.

Its use in pregnancy should be avoided, unless the expected therapeutic benefits outweigh the possible risks. **BS** should not be used in nursing mothers.

BS is contraindicated in premature and newborn infants within 1 to 2 months of birth because of the risk of producing kernicterus.

Warning and Precautions

To prevent cumulation in the blood of patients with impaired renal function, the dosage should be reduced or the interval between doses prolonged. The plasma drug concentrations should be monitored in such patients.

Regular blood counts are advisable whenever **BS** is given for prolonged periods. If a significant reduction in the count of any formed blood element is noted, **BS** should be withdrawn.

There should be adequate fluid intake to maintain urinary output at all times.

Treatment must be discontinued immediately if skin rash appears.

Use with caution in patients with G-6PD deficiency.

Avoid use in elderly patients as there is an increased risk of adverse reactions, in particular blood disorders and adverse renal/hepatic reactions.

As with all broad spectrum antimicrobials, there may be possible overgrowth of non-susceptible organisms. Should this occur treatment should be discontinued and appropriate therapy instituted.

Thrombocytopenia

Sulphamethoxazole/trimethoprim-induced thrombocytopenia may be an immune-mediated disorder. Severe cases of thrombocytopenia that are fatal or life threatening have been reported. Thrombocytopenia usually resolves within a week upon discontinuation of sulphamethoxazole/trimethoprim.

Haemophagocytic lymphohistiocytosis (HLH)

Cases of HLH have been reported very rarely in patients treated with sulfamethoxazole/trimethoprim. HLH is a life-threatening syndrome of pathologic immune activation characterised by clinical signs and symptoms of an excessive systemic inflammation (e.g. fever, hepatosplenomegaly, hypertriglyceridaemia, hypofibrinogenaemia, high serum ferritin, cytopenias and haemophagocytosis). Patients who develop early manifestations of pathologic immune activation should be evaluated immediately. If diagnosis of HLH is established, co-trimoxazole treatment should be discontinued.

Respiratory toxicity

Very rare, severe cases of respiratory toxicity, sometimes progressing to Acute Respiratory Distress Syndrome (ARDS), have been reported during co-trimoxazole treatment. The onset of pulmonary signs such as cough, fever and dyspnoea in association with radiological signs of pulmonary infiltrates, and deterioration in pulmonary function may be preliminary signs of ARDS. In such circumstances, co-trimoxazole should be discontinued and appropriate treatment given.

Fatalities associated with the administration of sulphonamides and trimethoprim, either alone or in combination, have occurred due to severe reactions, including Stevens-Johnson syndrome, toxic epidermal necrolysis and other reactions. The drug should be discontinued at the first appearance of skin rash or any sign of adverse reaction.

Side Effects

At the recommended dosage **BS** is usually well tolerated. The reported adverse reactions correspond to those of a sulphonamide of moderately low toxicity. The more common side effects are nausea, vomiting and skin rashes. The less common are isolated cases of Stevens-Johnson and Lyell's syndromes; stomatitis, glossitis and mouth ulcers.

Haematological changes have been reported, the majority being mild, asymptomatic and reversible on withdrawal of the drug.

The changes mainly took the form of leucopaenia, neutropaenia and thrombocytopenia. Very rarely, agranulocytosis, megaloblastic anaemia and purpura occurred.

Drug Interactions

Cotrimoxazole may enhance the effect of concurrently administered phenytoin and oral anticoagulants e.g. warfarin. It may also enhance the effect of hypoglycaemic drugs.

Overdosage

In case of an overdose, perform gastric lavage. Force fluids orally or parenterally if renal function is normal to promote excretion. Both components are dialysable and dialysis should be considered in extreme overdosage in patients with impaired renal function.

Calcium folinate 3-6 mg I.M. for 5-7 days is an effective antidote for trimethoprim poisoning. Adrenal steroids may be given if jaundice develops.

Shelf-life

The expiry date is indicated on the packaging.

Presentation

White, oval tablets in blister pack of 1000's.

Storage

Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN
JAUHI DARI KANAK-KANAK

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Manufacturer and Product Registration Holder (In Malaysia)
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Internal code

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